



Clinical Session Report

GhaithAI · AI-Assisted Clinical Decision Support

● DRAFT · PENDING APPROVAL

Session Date: 28 Jun 2026

Report ID: 5FD67EFF

GhaithAI CLINICAL SUPPORT SYSTEM - NOT A DIRECT MEDICAL DIAGNOSIS. THIS AUTO-GENERATED REPORT IS FOR LICENSED CLINICIAN REVIEW ONLY. DO NOT DISCLOSE TO PATIENT WITHOUT CLINICAL INTERPRETATION.

Patient data processed under explicit consent for AI-assisted clinical support, recorded in system logs at the start of the session.

ATTENDING CLINICIAN

Clinician: Dr. Hossam Salama

Specialty: Couples & Family

PATIENT DEMOGRAPHICS & BASELINE

Name: Muhammed Abdeldayem

Stress Level: Moderate

Gender: Male

Sleep Quality: Excellent

Age: 24

Prior Therapy: Yes

Session Type: Voice

Current Meds: Yes

Primary Intake Concern: Relationship Problems, Depression

RISK ASSESSMENT

Overall Risk Tier: **MEDIUM**

Suicidal Ideation: **IDENTIFIED**

DETAILS

IDENTIFIED — Patient stated "يفكر انتحر" — suggesting passive ideation. No plan, intent, timeline, or means access was identified in the transcript; clinical confirmation by the treating clinician is required. [STATED]

NARRATIVE & PROTECTIVE FACTORS

No self-harm ideation was identified in the transcript; clinical confirmation by the treating clinician is required. [NO SELF-HARM IDENTIFIED IN TRANSCRIPT] No harm-to-others ideation was identified in the transcript; clinical confirmation by the treating clinician is required. [NO HARM-TO-OTHERS IDEATION IDENTIFIED IN TRANSCRIPT] No protective factors were identified in this session. [NO PROTECTIVE FACTORS IDENTIFIED IN TRANSCRIPT] Clinical indicators are consistent with passive suicidal ideation and work-related distress, with no active plan or intent identified in the transcript. Limited protective factors are identified in this session. Urgent further assessment is suggested.

REQUIRED CLINICIAN ACTIONS

ORANGE ESCALATION: PATIENT IS CLASSIFIED AS MEDIUM SUICIDE/SELF-HARM RISK. CLINICIAN ACTIONS REQUIRED:

1. Complete collaborative safety plan and review emergency contacts.
2. Schedule close follow-up appointment within 72 hours.
3. Provide patient with crisis contact numbers (16394 / 080-8888332).

CLINICAL SUMMARY (SOAP)

S — Subjective

Patient reports recent onset of suicidal ideation, stating "يفكر انتحر يعني مؤخرأ". He describes work-related distress, expressing a desire not to work ("مش عايز اشتغل") and referencing "سوء العمل". No further details on duration, triggers, or associated symptoms were elicited due to the brief and fragmented session.

O — Objective

Mental status exam limited by transcript. Speech appears coherent but brief. Mood presents with features consistent with depressed mood given reported suicidal content. Thought process appears linear, with suicidal ideation as primary content. No psychotic symptoms, NSSI, or homicidal ideation were reported. Insight and judgment are not fully assessable. Risk tier: MEDIUM due to reported passive SI without plan or intent.

A — Assessment

Presentation is consistent with features of a mood disturbance, likely reactive to occupational stress. Passive suicidal ideation is reported by the patient but without active plan or intent identified in the transcript. Differential



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includes adjustment-related distress features with depressed mood versus depressive spectrum features. Further assessment is suggested to clarify clinical picture, including evaluation of anhedonia, sleep, appetite, and duration.

P — Plan

1. DIAGNOSTIC: Administer PHQ-9 and C-SSRS to assess depression severity and suicide risk. 2. THERAPEUTIC: Supportive therapy focusing on coping with work stress; explore protective factors. 3. SAFETY: MEDIUM risk — schedule 24-hour follow-up; review safety plan; provide crisis resources. 4. PSYCHOEDUCATION: Discuss connection between stress and mood, and importance of seeking help. 5. FOLLOW-UP & REFERRAL: Next session within 1 week; consider referral to occupational counseling if indicated.

CHIEF COMPLAINT

Primary Concern

Suicidal ideation. [STATED]

Duration

Recent (exact onset not specified). [INFERRED: from "مؤخراً"]

Episode Type

NOT_MENTIONED

Secondary Complaints

[VERBATIM: "بفكر انتحر يعني مؤخراً"] Work-related distress, expressed as "مش عايز اشتغل". [STATED]

HISTORY OF PRESENTING ILLNESS (HPI)

Narrative

Patient reports recent onset of suicidal ideation in the context of work-related stress and dissatisfaction. No further details on course, relieving factors, or associated symptoms were elicited. The session was brief and fragmented.

Functional Impact

Work/Academic: Impaired — expressed desire not to work. [STATED] Sleep: [NOT MENTIONED] Appetite: [NOT MENTIONED] Energy/Psychomotor: [NOT MENTIONED] Interpersonal: [NOT MENTIONED] Self-care: [NOT MENTIONED] Cognition: [NOT MENTIONED]

Past Psychiatric & Medical History

NOT_MENTIONED

Current Medications & Substances

NOT_MENTIONED

MENTAL STATE EXAMINATION (MSE)

Appearance & Behavior

NOT_ASSESSABLE

Speech, Mood & Affect

Coherent, brief, with hesitancy ("أوه", "يعني"). [INFERRED: from transcript] Presents with features consistent with depressed mood, as suggested by the patient's reported suicidal ideation. [INFERRED: from stated SI] [NOT ASSESSABLE FROM TRANSCRIPT]

Thought Process & Content



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Thought form appears linear and goal-directed. [INFERRED: from coherent responses] Thought content includes reported passive suicidal ideation and work-related hopelessness. [STATED]

Perception & Cognition

[NOT MENTIONED] Cognition appears alert and oriented (implied by interaction). [INFERRED: from conversation]

Insight & Judgement

Insight appears possibly limited; patient acknowledges distress but no further reflection is observed. [INFERRED — LOW CONF: limited data] [NOT ASSESSABLE FROM TRANSCRIPT]

CLINICAL FORMULATION

Narrative

No predisposing factors were identified. Precipitating factors include work-related stress and dissatisfaction ("سوء العمل", "مش عايز اشتغل"). Perpetuating factors are insufficiently captured in the transcript. No protective factors were identified. Patient presents with features consistent with recent passive suicidal ideation in the context of occupational stress. No prior psychiatric history or protective factors are mentioned. The limited data suggests features consistent with a reactive mood disturbance, but further evaluation is suggested to assess for depressive spectrum features.

Differential Considerations

1. Adjustment-related distress features with depressed mood
2. mood-related presentation warranting evaluation for depressive spectrum features
3. occupational stress reaction features